

Distinguish arteritic from non-arteritic AION

GCA symptoms with optic nerve ischaemia demand urgent systemic treatment.

AION comparison

Feature	NAION	AAION
Context	Vascular risks; crowded disc	Giant cell arteritis
History	Painless loss, often on waking	Profound loss; GCA symptoms
Disc	Hyperaemic swelling	Often chalky pale swelling
Action	Assess risks and fellow eye	Urgent systemic steroids

Clinical meaning

An inferior altitudinal defect can occur in either form. Ask about temporal headache, scalp tenderness and jaw claudication. Suspected arteritic disease needs urgent high-dose systemic treatment and GCA investigation to protect the fellow eye. A negative temporal artery biopsy cannot alone dismiss a convincing presentation because skip lesions occur.

A bilateral central problem may be toxic or inherited

Symmetry can reduce the RAPD despite genuine optic nerve dysfunction.

Two bilateral central patterns

Feature	Ethambutol	LHON
Context	TB treatment; renal risk	Young patient; maternal history
Vision	Bilateral blur and colour loss	Sequential bilateral central loss
Assessment	Acuity and colour testing	Phenotype and genetic testing
Response	Stop drug if toxicity develops	Assess and support visual function

Clinical meaning

Symmetric bilateral nerve impairment can produce little RAPD, and early discs may look normal. With suspected ethambutol toxicity, stop the drug promptly and involve the treating team to review TB therapy. LHON has mitochondrial maternal inheritance. Disc pallor reflects established nerve damage rather than the only diagnostic stage.